

HHS “Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care”

February 2026

This response leverages the governance, targeted data collection, deviation logging, and rapid learning cycles central to Standardized Clinical Assessment and Management Plans (SCAMPs), and maps them to AI evaluation, regulation, reimbursement, and research needs in clinical care.

Q1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Response:

- Evidence and generalizability gaps: Many AI tools lack prospective, real-world evidence across diverse settings; performance drifts over time without structured monitoring. SCAMPs’ targeted outcomes and deviation logging provide a blueprint for continuous, site-specific learning and generalization testing.
- Integration and workflow friction: Limited point-of-care integration (CDS Hooks/SMART on FHIR), fragmented EHR data, and poor usability impede adoption. SCAMPs emphasize CDS integration and clinician-guided pathways for practical use.
- Governance and liability uncertainty: Unclear accountability for “non-device” AI (e.g., clinical decision support) and continuous-learning systems. SCAMPs’ multidisciplinary committees provide an actionable governance model for oversight, deviation review, and iterative improvements.
- Payment and ROI ambiguity: Lack of clear reimbursement or recognized value signals for AI-enabled services and care redesign; hard to sustain QI cycles without payment alignment. SCAMPs demonstrate resource optimization through iterative refinement, but need payment models that reward measured outcome improvements.
- Data access and quality: Incomplete, heterogeneous data; limited access to benchmarking datasets and shared evaluation tools; difficulty capturing rationales behind deviations. SCAMPs stress structured inputs and rationale capture to improve data fidelity and learning.
- Trust, transparency, and safety: Insufficient post-deployment monitoring, fairness audits, and human-centered evaluation; clinician concerns about alert fatigue and explainability. SCAMPs’ rapid learning cycles and explicit documentation of diversions can build trust through transparent improvement.

Q2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited?

Response:

- Create an AI-in-clinical-care evidence pathway:

- o Establish a “coverage with evidence development” track for non-device AI-enabled clinical services that commit to SCAMPs-like targeted data collection, deviation logging, and iterative improvement. Align CMS payment with measurable outcome/resource improvements.
 - o Expand New Technology Add-on Payment-like mechanisms for AI-enabled interventions demonstrating improved outcomes/costs in real-world learning cycles.
- Certify AI-enabled CDS and learning loops:
 - o Update ONC health IT certification criteria (e.g., 45 CFR Part 170) to include AI transparency, ongoing post-deployment monitoring, fairness, calibration drift checks, and integration hooks (CDS Hooks, SMART on FHIR, FHIR PlanDefinition).
- Clarify privacy, security, and secondary use:
 - o Provide guidance under HIPAA (e.g., 45 CFR Part 164) on responsible secondary use for QI and learning health systems; promote privacy-preserving analytics/federated learning for SCAMPs-like cycles.
- Institutional governance standards:
 - o Encourage Conditions of Participation-level expectations (e.g., 42 CFR Part 482) for AI safety governance: multidisciplinary oversight, deviation/rationale capture, and real-world performance reporting (akin to SCAMPs committees).
- Programmatic supports:
 - o Launch HHS grants/CRADAs for “SCAMPs+AI” pilots across varied conditions/settings using FHIR Clinical Reasoning, PlanDefinition, and GuidanceResponse to run targeted experiments and share updated pathways.

Q3. For non-medical devices, what novel legal and implementation issues exist and what role, if any, should HHS play to address them?

Response:

- Issues:
 - o Allocation of liability when AI advice is advisory and clinicians deviate or comply; continuous-learning updates changing behavior over time.
 - o Vendor lock-in, data rights to rationales and outcomes, and responsibilities for monitoring/calibration drift.
 - o Privacy risks in rationale capture and free-text deviation documentation; bias and fairness obligations for “learning” tools.
- HHS roles:
 - o Publish model contractual clauses for AI in clinical care covering auditability, drift monitoring, fairness testing, data/rationale rights, and update governance aligned with SCAMPs deviation logging.
 - o Create guidance for shared accountability frameworks (clinician, institution, vendor) tied to documented deviations and outcomes, mirroring SCAMPs committees’ oversight.

- o Issue privacy/security best practices for capturing rationales (structured inputs, de-identification, access controls) and human-centered evaluation.

Q4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), and should HHS support them?

Response:

- Methods:
 - o Pre-deployment: External validation across sites; fairness and subgroup performance checks; robustness to missing/noisy data; simulated workflow testing; human factors/usability studies.
 - o Post-deployment: SCAMPs-like targeted outcomes with deviation logging; stepped-wedge trials; A/B testing; causal inference (propensity, synthetic controls); calibration drift monitoring; clinician override analysis; alert burden tracking.
 - o Metrics: Outcome impact (clinical endpoints); resource use; safety (false positive/negative harm indices); equity (disparate impact); usability (time-on-task, override rates); economic (cost per improved outcome).
- HHS support:
 - o Fund real-world evaluation testbeds that embed SCAMPs' learning cycles and share standardized metrics/benchmarks; support prize competitions for reproducible post-deployment monitoring toolkits.
 - o Encourage registry reporting of performance, deviations, and rationales using FHIR Clinical Reasoning artifacts.

Q5. How can HHS best support private sector activities (accreditation, certification, industry-driven testing, credentialing) to promote innovative and effective AI use?

Response:

- Establish an AI Clinical Care Trust Mark: A voluntary accreditation signaling adherence to SCAMPs-like governance (multidisciplinary oversight, deviation/rationale capture, iterative updates), robust evaluation, fairness audits, and transparent performance reporting.
- Seed neutral testing labs: Fund third-party labs to run standardized cross-site evaluations using shared datasets and FHIR-integrated workflows (CDS Hooks/SMART apps), and publish comparative benchmarking.
- Credentialing and training: Support curricula for clinicians and quality leaders on AI oversight, SCAMPs methodology, and human-centered evaluation; recognize institutions with mature AI governance.
- Promote industry consortia: Convene payers, providers, EHR vendors, and AI developers to agree on "learning health system" reporting packages (deviations, outcomes, fairness) aligned to SCAMPs.

Q6. Where have AI tools met/exceeded expectations and where fallen short? What novel AI tools have greatest potential?

Response:

- Met/exceeded:
 - Imaging triage/diagnostics (e.g., prioritization, fracture/retinopathy detection) with workflow-integrated CDS; documentation productivity tools; some operational AI (e.g., scheduling) improving throughput.
- Fallen short:
 - Generalization across sites/patient populations; alert fatigue in CDS; limited transparency; sepsis/risk scores with high false positives and equity concerns without robust monitoring.
- High-potential tools:
 - AI+SCAMPs clinical pathways: Tools that embed PlanDefinition-based recommendations, capture deviations/rationales, and update iteratively.
 - Causal inference and real-time analytics dashboards for targeted outcomes, enabling rapid learning cycles at the point of care.
 - Safety and equity monitors (drift/fairness) integrated into EHRs; NLP to structure rationale capture; federated learning for multi-site improvement.

Q7. Which roles/decision makers have the most influence on adoption? Primary administrative hurdles?

Response:

- Influencers:
 - CMIO, CMO, Chief Nursing Officer, Pharmacy & Therapeutics/Value Analysis committees, Quality & Safety leaders, Privacy/Compliance, Security/IT, Finance, and Patient/Family advisory councils. SCAMPs governance mirrors this multidisciplinary structure.
- Administrative hurdles:
 - Procurement and contracting (risk/liability, data rights); integration costs; EHR workflow fit; governance/approval cycles; training and change management; performance monitoring resources; reimbursement/ROI clarity. SCAMPs can streamline approval via targeted outcomes and deviation-based learning commitments.

Q8. Where would enhanced interoperability widen opportunities, fuel research, and accelerate AI? Consider data types, standards, benchmarking tools.

Response:

- Priorities:
 - Standards: Broader adoption of FHIR Clinical Reasoning (PlanDefinition, ActivityDefinition, GuidanceResponse), CDS Hooks, and SMART on FHIR apps to deliver SCAMPs-aligned recommendations and capture deviations/rationales.
 - Data types: Outcomes, orders, observations, imaging, genomics, device telemetry, SDOH, care plans; structured rationale inputs and deviation flags as first-class data.

- o Benchmarking tools: Public test datasets and reference implementations for post-deployment monitoring (calibration/fairness), drift detection, and causal inference; shared dashboards and report formats.
- o Multicenter datasets and collaboration (especially for rare diseases):
 - Harmonize data capture and phenotyping across sites using FHIR profiles, PlanDefinition/ActivityDefinition, and standardized case definitions so targeted outcomes and deviation/rationale capture are consistent across institutions.
 - Support privacy-preserving analytics (federated learning, secure multiparty computation) to enable cross-site evaluation and improvement without centralizing raw patient data.
 - Leverage common data models (e.g., OMOP, PCORnet) alongside FHIR to enable reproducible cohort definition, benchmarking, and pathway performance comparisons in low-prevalence populations.
- Actions:
 - o Expand USCDI to include “rationale for deviation” and “targeted outcomes” elements; promote interoperable pathway updates (FHIR PlanDefinition) across sites.
 - o Catalyze multicenter learning networks for rare diseases:
 - Provide model DUAs/BAs and governance templates to streamline cross-institution collaboration, IRB reciprocity, and SCAMPs-like targeted data collection and deviation logging across sites.
 - Fund reference implementations that integrate FHIR Clinical Reasoning artifacts with federated analytics for multicenter evaluation, including drift/fairness monitoring and causal inference at low sample sizes.
 - Expand USCDI to include rare disease-relevant elements (standardized phenotype descriptors, natural history measures, and “rationale for deviation” fields) to improve data quality and comparability across centers.
 - Align with existing NIH/NCATS rare disease consortia and registries to ensure interoperable data exchange and shared benchmarking that can directly inform SCAMPs pathway updates.

Q9. What challenges do patients/caregivers want addressed and what concerns do they have?

Response:

- Desired benefits:
 - o Faster, more accurate diagnosis and triage; medication safety; care coordination and navigation; reduced burden in chronic disease management; equitable access and outcomes. SCAMPs’ iterative improvement focuses on measurable outcomes that matter to patients.
 - o State-of-the-art, optimal care: Patients expect care plans and recommendations to reflect the most current, best-available evidence and

expert consensus. AI-enabled, SCAMPs-aligned pathways can help ensure care stays up to date by continuously incorporating new data, guideline changes, and documented clinician innovations (via deviation/rationale capture), with transparent update histories.

- Concerns:
 - Privacy and data reuse; bias and fairness; transparency/explainability; overreliance on automation; consent and opt-out options; recourse for errors. SCAMPs' explicit logging of deviations and rationales enables patient-centered accountability and continuous improvement.
- Actions:
 - Require patient-facing explanations of how AI is used; offer opt-out where feasible; include patient councils in SCAMPs/AI governance committees; publish outcomes and equity metrics in plain language.
 - Provide patient-facing assurances that recommendations are current: Display pathway "last updated" dates, summarize recent evidence changes, and explain how SCAMPs/AI-driven learning cycles keep care aligned with the latest knowledge.

Q10. Specific areas of AI research HHS should prioritize to accelerate adoption; a) published findings, b) costs/benefits literature?

Response:

- Research priorities:
 - Learning health systems methods: SCAMPs-aligned real-world evaluation frameworks (targeted outcomes, deviation/rationale capture, rapid iteration), federated learning and privacy-preserving analytics.
 - Robustness and fairness: Drift detection, re-calibration, subgroup performance, bias mitigation; human-centered design to reduce alert fatigue and increase safe overrides.
 - Causal inference at scale: Tools to attribute impact of AI-guided care pathways on outcomes and resource use; stepped-wedge/A/B designs integrated into EHR workflows.
 - Interoperability toolkits: FHIR Clinical Reasoning and CDS Hooks reference implementations; PlanDefinition authoring/sharing; standardized reporting packages.
 - Economic evaluation: Real-world cost-effectiveness analyses of AI-enabled SCAMPs, including downstream benefits (complication avoidance) and administrative savings.
- a) Published findings:
 - Literature shows mixed impact: success in specific domains (e.g., imaging, documentation) and limitations in generalization, fairness, and workflow fit—underscoring need for continuous, SCAMPs-like monitoring and iterative improvement.
- b) Costs/benefits:
 - Studies increasingly consider total value: clinical outcomes, resource utilization, safety, equity, clinician burden, and operational efficiency;

strongest cases pair payment incentives with monitored improvements over time, which SCAMPs operationalize via targeted outcomes and iterative cycles.

How SCAMPs can serve as a backbone for AI in clinical care

- Governance: Multidisciplinary committees oversee pathway design, evidence review, and iterative updates; this maps directly to AI oversight and accountability.
- Targeted data collection: Focused “plausible outcomes” and structured rationale capture enable high-signal learning and pragmatic evaluation of AI recommendations.
- Deviation logging: Recording positive/negative diversions with rationale identifies innovation and safety signals, informing pathway and model updates.
- Informatics integration: EHR cohort discovery, CDS Hooks/SMART on FHIR apps, structured inputs/NLP, AI-based causal inference, dashboards, and FHIR PlanDefinition sharing support scalable deployment and continuous improvement.
- Outcomes: Published SCAMPs have demonstrated applicability across conditions/settings and can reduce resource use through rapid, evidence-informed iteration—providing a practical template to evaluate AI’s value.

Citations:

Farias M, Alexander PMA, Geppert JJ, Glavin P, Ramirez-Mendoza H, Casale P, Jenkins K. Standardized Clinical Assessment and Management Plans Revisited: Lessons Learned from a Decade of Implementation, Health Affairs Scholar, 2026;, qxag036, <https://doi.org/10.1093/haschl/qxag036>

- HHS: Request for Information—Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care (2025-23641).